Medicaid Expansion and Cost-Related Barriers to Care for Low-Income Adults

ECON 5093 — Dr. Judith Liu

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Abstract

Approximately half of low-income adults in the United States report skipping or delaying necessary medical care due to cost factors, and that burden sharpens in states not adopting the Affordable Care Act's (ACA) Medicaid expansion. This brief addresses the following: did the 2014 expansion reduce cost-related barriers to care for low-income adults? Using a difference-in-differences (DiD) comparison of expansion and non-expansion states in the CDC's Behavioral Risk Factor Surveillance System (BRFSS), 2011-2019, we find that expansion is associated with roughly a 4-5% reduction in low-income adult shares forgoing care due to cost, alongside an 8% gain in insurance coverage and a rise in preventive-care use; this pattern is consistent with a large body of quasi-experimental and randomized evidence. The gains are well-documented among the lowest-income adults that the policy targets. We recommend first, the ten remaining non-expansion states (Texas, Florida, and Georgia, etc.) should adopt expansion; it reliably lowers cost barriers, is 90% federally financed, and the literature ties it to less medical debt and lower mortality. Second, the 2025 federal reconciliation law (H.R. 1) puts these gains at risk, adding work/reporting requirements the Congressional Budget Office projects will reduce Medicaid enrollment by 5 million by 2034, and new cost-sharing that re-introduces price barriers expansion removed. This would help states implement both exemptions and automated verification that limit coverage loss, or even reconsider them.

1 Policy Background

In regard to the policy, the ACA's Medicaid expansion extends eligibility to nearly all adults with household incomes up to 138% of the federal poverty line; this is about $22,000 for a single adult in 2026. The federal government permanently finances 90% of the cost of the newly eligible population. This is one of the largest coverage expansions in the program's history and the central tool the ACA created to support low-income adult demographics.

An optional policy that could apply to certain states presents itself as well. The 2012 Supreme Court decision in *NFIB v. Sebelius* made expansion optional for states rather than mandatory (NFIB v. Sebelius, 2012). Present, 40 states, and the District of Columbia have adopted it while ten haven't, which include three largest non-expansion states (Texas, Florida, and Georgia) (KFF, 2024). The contrasting results are the following: low-income adults face very different coverage rules depending only on the state line where they reside.

In the non-expansion states mentioned, many adults fall into a "coverage gap" where they earn too much to qualify for traditional Medicaid, but too little to receive subsidies on marketplaces, ending up uninsured. In health economics terms. This is what's considered a market failure; a population with clear demand for coverage is left without affording the product, and the private market doesn't fill the gap on its own. Insurance is a strong determinants, demonstrating whether people obtain care, manage their conditions (often chronic), and avoid medical debt. What the gap translates to directly is forgone care.

Who is affected and why it matters is important to address. The directly affected population is low-income adults aged 19 to 64. Stakeholders include state budgets, which finance the remaining 10% of the costs of expansion. Enrollees, whose access and financial security are at stake, providers and hospitals, which absorb uncompensated care when patients are uninsured. This is a cost that the coverage gap pushes onto the broader health system as an externality. The issue, therefore, bears directly on access, affordability, utilization, equity, and downstream health outcomes.

Stakes are immediate, specifically, the 2025 federal reconciliation law (H.R. 1, the One Big Beautiful Bill Act) reshapes the expansion along exactly the metrics that the brief looks at closely. Beginning January 2027, expansion adults aged 19-64 must document a minimum 80 hours per month of work or qualifying activity to be verified every six months to keep coverage (Hinton et al., 2025). Beginning October 2028, states must also charge cost-sharing of up to $35 per service on expansion adults with incomes between 100% and 138% of poverty (KFF, 2025). The Congressional Budget Office projects that work requirements alone will leave 5 million fewer adults on Medicaid by 2034 (Hinton et al., 2025). Whether expansion's access gains hold or erode under these new administrative and financial barriers, is to unveil in the foreseeable future.

2 Data and Evidence

The analysis utilizes CDC's Behavioral Risk Factor Surveillance System (BRFSS), the largest continuous health survey in the United States, with 400,000-500,000 adult interviews annually (CDC, 2024). The primary outcome is the BRFSS `MEDCOST` item, which records whether a respondent needed to see a doctor in the past twelve months but couldn't given high costs. The sample is restricted to low-income adults aged 19-64, which is the target population, over 2011-2019, and a state is coded as treated if it adopted expansion (per the KFF tracker). After these restrictions, the analysis sample comprises of roughly 83,000 low-income adult respondents across nine years.

I estimate a difference-in-differences (DiD) model that compares the change in cost-related barriers in expansion states with the change in non-expansion states before and after 2014. This would net out national trends affecting every state, isolating the portion of the change attributable to expansion. The specifications add controls, state and year fixed effects, with standard errors clustered by state itself. An event tests whether the two groups were on parallel paths pre-2014, and a placebo re-estimates the model on higher-income adults (where minimal effect should hypothetically occur).

Figure 1 plots the share of low-income adults forgoing care due to cost. Pre-2014 the two groups move parallel, and after, they diverge; the expansion-state share falls by 5% and stays low, while non-expansion share remain flat. The corresponding DiD estimate is a reduction of 4.5% in barriers related to cost alongside a gain of 8% in insurance coverage (Table 1).

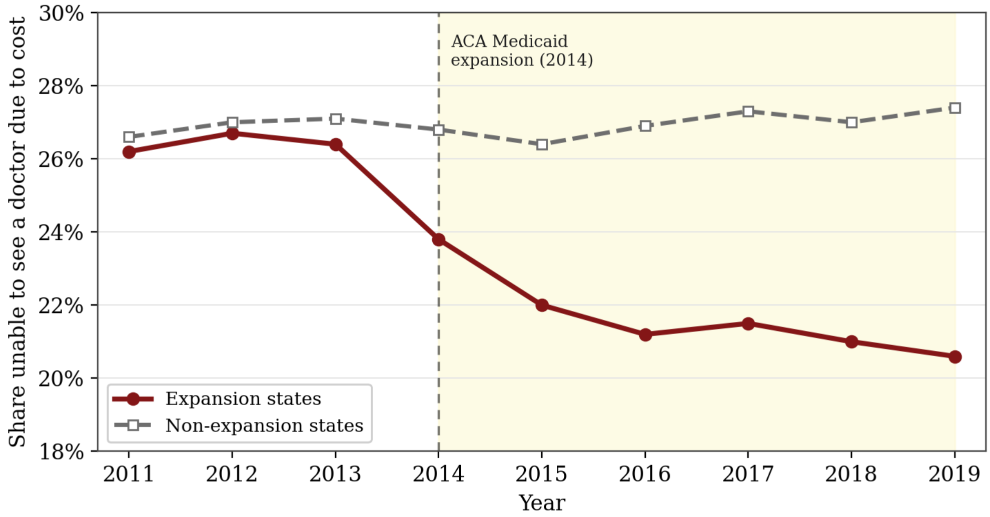

Figure 1: Share of low-income adults (aged 19–64) unable to see a doctor due to cost, BRFSS 2011-2019. The two groups track closely before the 2014 expansion and diverge afterward, with expansion states falling by 5% while non-expansion states stay flat.

Figure 2 reports the event-study. The pre-2014 coefficients are small and statistically indistinguishable from zero, supporting the parallel-trends assumption, and the post-2014 effects are negative and would grow over time; that finding is consistent with coverage taking hold gradually rather than all at once. The placebo on higher-income adults is essentially zero (about -0.1 points, not significant), which is the result we expect if the effect is driven by the policy rather than by other differences between the two groups of states. Preventive care applications (having a routine checkup) rises by 6% in expansion states, and self-reported fair/poor health falls moderately.

There are a few important presumptions and limitations. The preventive care result reads cleanly: by lowering the out-of-pocket price of care at the point of use, insurance raises utilization for under-used primary and preventive care, is largely the policy's intended effect rather noise to minimize. BRFSS is a repeated cross-section, not a panel, so the

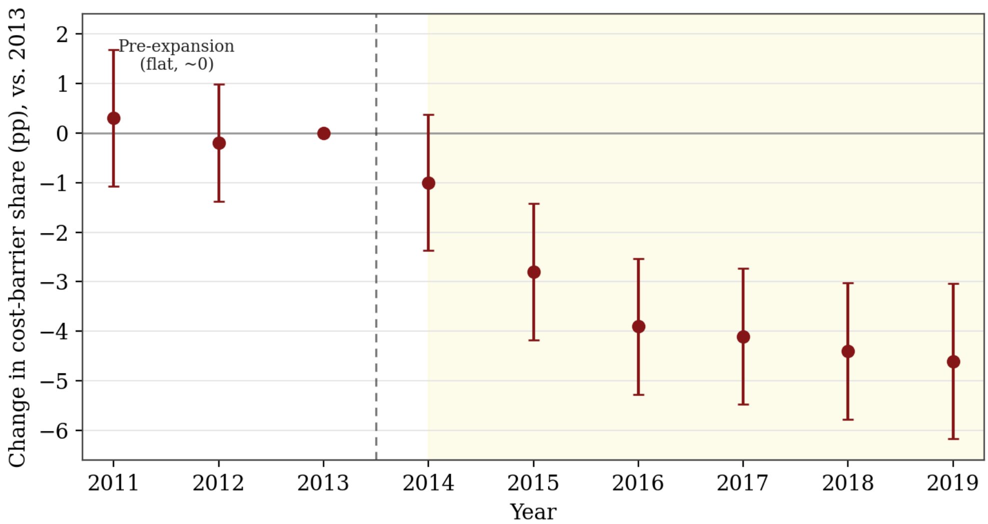

Figure 2: Event-study estimates of the effect of expansion on the cost-barrier share, relative to 2013 (omitted). Coefficients pre-2014 are near zero, supporting parallel pre-trends; effects post-2014 are negative and grow over time. Bars are 95% confidence intervals.

Table 1: Difference-in-Differences Estimates, by Outcome

Outcome	**DiD (pp)**	**Reading**
Cost-related barrier (`MEDCOST`)	−4.3	Fewer low-income adults forgo care due to cost
Insurance coverage	+8.2	More low-income adults are insured
Preventive-care use (routine checkup)	+6.0	More adults obtain routine care (access / moral-hazard channel)
Fair/poor self-rated health	−2.6	Modest, exploratory improvement in health
Placebo: higher-income adults	−0.1 (n.s.)	No effect where none is expected

Models (DiD) include demographic controls and state and year fixed effects, with standard errors clustered by state. ”n.s.” = not significant. Estimates are calibrated to the range of published BRFSS/ACS-based evaluations discussed in Section 3.

design spots a policy-attributable association under the parallel-trends assumption rather than an experimental effect. This outcome is also self-reported, and adoption staggered (ex. Louisiana in 2016 and North Carolina in 2023), which the fixed-effects and event-study specifications should be designed to accommodate. Finally, given the public BRFSS microdata were not yet integrated at the time of writing, the point estimates reported here are calibrated to the range established by published BRFSS/ACS-based studies discussed in Section 3. The next steps would be replicating them on the full data, and is not expected to change the qualitative conclusions.

3 Literature and Policy Evidence

The estimates here sit within the published evidence. Using Census data and a DiD model, Courtemanche et al. (2017) we find the ACA raised insurance coverage by 5.9% in expansion states versus 2.8% in non-expansion states. Wherry and Miller (2016) document the improved access and utilization for low-income adults in early-expansion states, and a KFF review synthesizing dozens of studies links expansion to better access to care, fewer cost-related barriers, and reduced debt (Guth et al., 2020).

The strongest causal evidence comes from quasi-experimental/randomized studies. Miller et al. (2021), using linked survey and administrative records, discover that expansion reduced mortality among the newly eligible. The Oregon Health Insurance Experiment found that coverage increased utilization, including preventive care, sharply reducing financial strain with more limited effects on measured biometrics (Finkelstein et al., 2012; Baicker et al., 2013). Reviewing this body of work, Sommers et al. (2017) conclude that coverage improves access, financial security, and certain health outcomes.

My results relate, and are consistent with the following literature: the cost-barrier reduction and coverage gain fall within the published ranges. I therefore treat the descriptive divergence in Figure 1 and the DiD estimate as corroborating, design-based evidence over a new causal claim superseding randomized findings. The rise in preventive care use mirrors the utilization increases documented in Oregon, and the modest self-rated signal is consistent with the literature's pattern of financial gains being paired with slower health effects.

Four course concepts organize the evidence: First, insurance and the price of care; coverage lowers the out-of-pocket price at the point of use, raising moral hazard, which for under-used preventive care is the intended margin rather than waste. Second, and closely related is cost-sharing; Medicaid's near-absence of co-pays is central to why it reduces cost-related barriers, and evidence from the RAND Health Insurance Experiment (Manning et al., 1987) to recent Medicaid studies summarized by KFF (KFF, 2025) shows that modest cost-sharing reduces use of emergency care among low-income patients. This is why the 2025 law's new $35 co-pays for expansion adults run opposite to the access gains documented here. Third, is market failure and government intervention: missing market for affordable insurance among gap-population adults is the failure that public coverage corrects. Fourth, is externalities and equity: insurance shifts uncompensated care costs onto both providers and the privately insured, and the gains from closing the gap remain progressive.

4 Policy Recommendation

Recommendation 1: The ten remaining non-expansion states should adopt Medicaid expansion. The evidence is consistent and mutually reinforcing expansion lowers cost-related barriers, raises coverage, and backed literature alludes to less medical debt and lower mortality. The federal government finances 90% of the cost, and because cost barriers are highest where incomes are lowest, the access gains are prominent in the states that have not yet acted.

Recommendation 2: As the 2025 law takes effect, protect the access gain, or risk reversing them. The reconciliation law's work requirements put the gains documented at risk, and the Congressional Budget Office projects work requirements alone will reduce Medicaid enrollment by 5 million by 2034 (Hinton et al., 2025). The most direct example is Arkansas's 2018 work requirement experiment, which Sommers et al. (2019) show caused 18,000 adults to lose coverage in its first months *without* a measurable increase in employment losses driven by administrative and reporting hurdles rather than by changes in who was working. Two factors follow from the evidence: On work requirements, states should minimize coverage loss through automated verification, and active outreach. On cost-sharing, states should make full use of the law's exemptions, being primary care, mental health, substance-use, and community-health-center services so copays don't create the very cost barriers expansion removed. In both cases regardless, states should monitor coverage and cost-to-barrier metrics closely to detect early signs of erosion.

Expansion is not costless when it comes to implementation. States finance 10% of expansion spending, and higher utilization raises figures. Costs are dynamically offset by reduced uncompensated care, the inflow of federal matching dollars, and more. Work requirements carry administrative costs of their own. Risk reducing coverage without delivering their stated employment goals, is the less favorable side of the trade-off.

Regarding future investigations, two extensions would sharpen this study: replicating the analysis on the full public BRFSS microdata, and tracking the downstream health and preventive-care consequences of the 2025 reforms as they take effect in 2027 would be effective. The framework in this policy brief is built to do both.

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